Water Care

Preliminary proposal to Global Affairs Canada
Submitted 27 April 2018

Synthetic training document. Water Care, Taps for Rights, Nimbara, Sundara and every organization, place and figure in this proposal are fictional. The layout follows the public Global Affairs Canada (GAC) preliminary proposal form used for the 2018 call "Partnerships for Her Voice, Her Choice". Content has been **deliberately altered to include gaps, errors and inconsistencies** so you can test whether your tool finds them. Dollar amounts are omitted on purpose.

Call for Preliminary Proposals: Partnerships for Her Voice, Her Choice

Preliminary proposal form (modelled on GAC form 2520E). Applicant's page count: 9 of 15.

Organization name and contact information

Organization name	Water Care
Initiative contact name	[Name withheld]
Position title	Head of Program Development
Country	Canada
Contact telephone and email	[Withheld for this exercise]

Eligibility requirements

A. Cost share

> Your organization must provide at least 10% of the total eligible direct project costs over the life of your project in cash and/or in kind. Please fill out the table below in dollar value. Third party contributions are considered an asset in the assessment of the proposal, but they cannot be counted as part of the applicant's cost share.

Total eligible direct project costs	Your organization's contribution (in cash)	Your organization's contribution (in kind)	Third party contributions
[Amount omitted for this exercise]	10% of total eligible direct project costs	None	Co-funding of hygiene kits by a national soap manufacturer in Sundara (to be confirmed)

B. Not-for-profit proposal

> Would your proposed project generate a profit for your organization or any other partner organization participating in the project?

No.

C. Locally-driven proposal

> Your organization must work with its own local office(s) or a local partner(s) in the country(ies) where your project would take place.

Water Care will deliver the project through its own country programmes, Water Care Nimbara and Water Care Sundara, working with two local implementing partners: the Association des Femmes Solidaires de Tenkori (AFST) in Nimbara and the Rural Development Support Network (RDSN) in Sundara. Details are in the Local Partners Information form (not included in this pack).

D. Relevant experience

See section 3.1.

E. Construction and refurbishment of existing structures

> Global Affairs Canada will NOT consider projects that include new construction activities. However, the Department will consider some refurbishment of existing structures. Refurbishment means any improvement to an existing structure adding no more than twenty-five percent (25%) of the existing footprint. Organizations that include refurbishment activities are required to demonstrate previous experience with such activities.

The Water Care Global Federation has more than 30 years of experience in new construction and rehabilitation of water supply systems (drilled wells, rainwater harvesting, gravity-fed systems and piped schemes) and many types of latrines. Water Care Nimbara has worked in water and sanitation construction for 15 years and Water Care Sundara for 12 years. Across the federation, our water and sanitation work has reached more than 25 million people.

In Taps for Rights, construction will focus on making existing school and clinic facilities usable for women and girls during menstruation. This may include:

- bringing a reliable water point to, or into, the latrine block (tap stands, rainwater tanks on latrine roofs, or a short pipe extension from an existing borehole);
- doors and locks for privacy and security, privacy walls, and ramps and handrails for accessibility;
- general repairs (masonry, roof sheeting, vent pipes) and emptying or repairing latrine pits so waste is kept away from groundwater;
- a private washing space with soap, hooks and a bin, and safe disposal of used pads (including small incinerators where needed).

Where a school has no usable sanitation at all, Taps for Rights will construct a new gender-sensitive latrine block with a dedicated menstrual hygiene room and an on-site tap, so that girls can change, wash and return to class.

Initiative summary

Title of the initiative (75 characters or less)	Taps for Rights: Water, Sanitation and Menstrual Health for SRHR
Proposed start date	2019-01-01
Proposed end date	2021-12-31
Total Global Affairs Canada funding requested	[Amount omitted for this exercise]

Sector(s)	14031 Basic drinking water supply; 14032 Basic sanitation; 14081 Education and training in water supply and sanitation; 12261 Health education; 15170 Women's rights organisations and movements, and government institutions
Country(ies)	Republic of Nimbara; Sundara
Signatory number 1	Water Care

1.0 Rationale for the initiative

1.1 Project summary

> Provide a brief overview of your proposed project (who, what, where, when, how). Explain how your project's ultimate outcome will contribute to poverty reduction.

Taps for Rights puts water where women and girls need it most: in the toilets and washrooms of schools and health centres. Without water on site, a private place to wash and a safe way to dispose of pads, girls miss school and women avoid clinics during menstruation. Menstruation is also the first moment many girls learn about their bodies, so it is a natural and accepted entry point for conversations about sexual and reproductive health and rights (SRHR).

Over three years, Water Care will work in Tenkori and Diafo districts of Kessara Region in Nimbara, and in Nadipur District of Rivana Province in Sundara, with local partners AFST and RDSN, youth engagement partner Northern Youth Exchange (Canada) and menstrual health specialists Bloom Health Collective (Netherlands). The project will:

1. help national and district governments include menstrual health, and the water and sanitation it depends on, in their policies, plans and budgets;
2. make water and sanitation facilities in 40 schools and 20 health centres usable during menstruation, and train teachers and health workers;
3. support local women and youth entrepreneurs to make and sell affordable menstrual products; and
4. run community awareness campaigns with women, men, girls and boys.

Studies show that girls in our target districts miss up to 20% of school days because of menstruation. When girls stay in school they marry later, have children later and earn more, which reduces poverty for their families and communities. Working across these three countries also lets programme teams learn from each other.

1.2 Innovation

> If applicable, identify how your project is innovative and how you will pilot, test and/or scale the innovative aspects.

Taps for Rights combines water engineering with menstrual health and SRHR, two fields that usually work apart. Water Care Sundara has run menstrual hygiene programmes in schools for several years; Water Care Nimbara will adapt that experience to the West African context through exchange visits and joint training. The project will also test small business models for reusable pads led by women's and youth groups. A design that puts a tap inside the menstrual hygiene room, rather than outside the latrine block, will be piloted in ten schools and compared with standard designs.

1.3 Alignment

> How does your project align with GAC's priorities, the Sustainable Development Goals (SDGs) and host country strategies and plans?

The project responds directly to Canada's Feminist International Assistance Policy, which commits Canada to ensure that school facilities include separate and appropriate washrooms and systems to help manage menstrual hygiene. It contributes to SDG 6 (water and sanitation), SDG 5 (gender equality), SDG 4 (inclusive and gender-sensitive education facilities) and SDG 3 (health). It is aligned with Nimbara's National Water and Sanitation Plan 2016-2030 and with the Rivana Province Education Sector Plan in Sundara, both of which call for gender-separated school toilets with water.

1.4 Support from the private sector

> Will your organization be using any GAC funding to obtain additional support for this project from the private sector?

No leveraging from private sector.

2.0 Managing for results

2.1 Preliminary theory of change

> Explain the assumptions, risks and contributing factors that link outputs to immediate outcomes, immediate to intermediate outcomes, and intermediate outcomes to the ultimate outcome. Show how your gender equality, human rights and environmental analyses inform each level.

Women and girls carry most of the water in rural Nimbara and Sundara, and they carry the burden of poor sanitation. At school and at health centres, a toilet without water, without a lock, or without a bin is not a toilet a menstruating girl can use. Silence and stigma around menstruation then limit what girls learn about their bodies and their rights. Two changes are needed: duty bearers must recognize menstrual health as a public responsibility, and women and girls must be able to use facilities and products that meet their needs.

Intermediate outcome 1100: Increased government and civil society responsiveness to the menstrual health needs of women and adolescent girls. Governments in both countries have committed to water and sanitation for all, but menstrual health rarely appears in education, health or water policies, and almost never in district budgets. The project will map gaps in national policies and guidelines (1111), support relevant ministries to close them (1112), and engage women leaders at national level as champions (1113). At district level, it will train officials and service providers (1121), map gaps in provision and feed them into planning and budgeting (1122), and run WASH at Work campaigns in markets and workplaces (1123). We assume that ministries will be open to discussing menstruation when it is framed as a water, sanitation and education issue.

Intermediate outcome 1200: Increased provision, awareness and utilization of menstrual health facilities and products in schools, healthcare centres and communities. Facilities in targeted schools and health centres will be rehabilitated and adapted so that water is available on site (1211). Teachers and health staff will be trained (1212) and students sensitized through peer learning (1213). Local supply chains for menstrual products will be mapped (1221) and women- and youth-led groups supported to make and sell products (1222). Community campaigns (1231) and community champions (1232) will reduce stigma. We assume schools and health centres will maintain facilities once rehabilitated, and that families will accept discussion of menstruation in schools.

Gender equality. The project fully integrates gender equality. Women and girls will be involved in design and in facility management committees.

Risks. Key risks are insecurity in parts of Kessara Region, seasonal flooding in Nadipur District, and resistance from some religious and traditional leaders. Mitigation: flexible site selection, flood-resilient designs, and early engagement of leaders.

Sustainability. District budgets for operation and maintenance, trained committees, and local businesses that sell products will sustain results after the project ends.

2.2 Logic model

Level	Results
Ultimate outcome 1000	Improved comprehensive sexual and reproductive health and rights (SRHR), with a focus on women and adolescent girls, through water, sanitation and menstrual health interventions in Nimbara and Sundara.
Intermediate outcomes	1100 Increased government and civil society responsiveness to the menstrual health needs of women and adolescent girls. 1200 Increased utilization of gender-sensitive WASH facilities and menstrual hygiene product supplies in schools, healthcare centres and communities.
Immediate outcomes	1110 Enhanced ability of national government officials to take menstrual health needs into account in national policies and guidelines. 1120 Enhanced ability of district and local authorities to take menstrual health needs into account in district plans and budgets. 1210 Enhanced ability of targeted schools and health centres to meet menstrual health needs, including reliable water on site. 1220 Enhanced ability of local private sector actors to deliver a sustainable supply of menstrual hygiene products. 1230 Improved awareness of community members of menstrual health as a foundation of SRHR and gender equality.
Outputs	1111 Gaps in existing policies and guidelines identified. 1112 Relevant national ministries (water and sanitation, education, health, gender) reached through advocacy and supported to address gaps. 1113 Women influencers at national level identified, engaged and informed to champion menstrual health. 1121 Local authorities and service providers trained on menstrual health. 1122 Menstrual health gaps in target districts mapped and used to inform planning and budgeting. 1123 Advocacy carried out for WASH at Work campaigns at workplaces and markets. 1211 Water points and WASH facilities in selected schools and health centres rehabilitated and adapted for menstrual hygiene. 1212 School and health centre staff trained on menstrual health and its lifecycle implications for SRHR. 1213 Conduct peer-to-peer sessions on menstrual health with adolescent girls and boys. 1221 Existing local supply chains and gaps for menstrual hygiene products mapped. 1222 Local businesses, women entrepreneurs and women- and youth-led groups supported to make and market menstrual hygiene products. 1231 Local awareness campaigns on menstrual health implemented through a variety of media. 1232 Community champions for menstrual health identified and trained.

3.0 Organization ability relevant to the initiative

3.1 Relevant applicant experience

> Provide two examples of projects you have managed in ODA-eligible countries. Both project examples must also demonstrate experience in sexual and reproductive health and rights, or other health-related programming.

Water Care has managed multi-country, consortium programmes funded by GAC for more than fifteen years and is part of the Water Care Global Federation, which shares learning on water, sanitation, hygiene, health, gender and education across its members.

Example 1

Initiative name	Clean Water, Healthy Communities (CWHC)
Approximate value	[Amount omitted for this exercise]
Country and location	Republic of Kanwe (fictional): two northern districts
Recipient	Water Care
Dates	2014-04-01 to 2019-03-31
Funding agency	Global Affairs Canada

CWHC is an integrated water, sanitation and public health programme delivering safe drinking water and gender-sensitive latrines in schools, health centres and communities, alongside hygiene and menstrual health promotion with health staff, nurses and midwives. A gender analysis at the start shaped a gender equality strategy developed with women, men, girls and boys. The programme has reduced diarrhoeal disease in target communities by 40% and all targeted schools now have menstrual hygiene facilities. Risks were managed through gender-sensitive indicators, regular monitoring visits and an environmental management plan for borehole drilling and latrine construction. Lesson learned: some communities resisted women taking decision-making roles on water committees; continued dialogue and behaviour change work were needed.

Example 2

Initiative name	Schools Water and Hygiene Programme (SWHP)
Approximate value	[Amount omitted for this exercise]
Country and location	Three East African countries: targeted districts
Recipient	Water Care
Dates	2012-06-01 to 2017-05-31
Funding agency	Global Affairs Canada

SWHP worked with nine local partners to improve water, sanitation and hygiene in primary schools, reaching more than 100,000 students and teachers. In the first year it became clear that menstrual hygiene was essential to keeping girls in school: at baseline, 44% of menstruating girls said they would not attend school during their period. Partners added washing rooms, pad disposal and training in making reusable pads. By Year 4 all partners had either a piped connection or a rainwater tank supplying the girls' washing room. Girls reported a more comfortable and dignified school day. Lesson learned: a minimum package of menstrual hygiene inputs for every school, including a reliable supply of pads and safe disposal, should be planned from the start.